

Bloodborne Pathogen Exposure and Needlestick / Sharps Injury

Policy Number	POL-EH-001	Owner / Department	Employee / Occupational Health · Environmental Health & Safety
Effective Date	2023-07-01	Version	5.1
Last Revised	2026-01-20	Next Review	2028-01-20
Approved By	Environmental Health & Safety Committee	Applies To	All employees, licensed providers, students, volunteers, and contract staff with potential occupational exposure

1. Purpose

To protect staff from occupational exposure to blood and other potentially infectious materials (OPIM) and to ensure a prompt, confidential post-exposure evaluation consistent with the OSHA Bloodborne Pathogens Standard (29 CFR 1910.1030).

2. Scope

Applies to any percutaneous injury (needlestick, cut with a contaminated sharp) or mucous-membrane / non-intact-skin contact with blood or OPIM occurring in the course of work.

3. Definitions

- **Exposure incident** — specific eye, mouth, other mucous membrane, non-intact skin, or parenteral contact with blood or OPIM.
- **Source individual** — the person whose blood or OPIM was involved in the exposure.
- **PEP** — post-exposure prophylaxis, medication started after a possible HIV exposure.

4. Immediate Actions (Employee)

IMPORTANT — Time-critical. HIV post-exposure prophylaxis is most effective when started as soon as possible — ideally within 2 hours — and is not recommended beyond 72 hours. The exposed employee must be evaluated by Employee Health within 2 hours of the exposure (go to the Emergency Department if Employee Health is closed).

1. Immediately wash the site with soap and water; flush mucous membranes and eyes with water or saline for 15 minutes.
2. Report the exposure to your charge nurse or supervisor right away — do not wait until the end of shift.
3. Present to Employee Health (or the ED after hours) within 2 hours for evaluation and, if indicated, PEP.
4. File an event report per POL-RM-013 before the end of the shift.

5. Policy — Source Testing and Evaluation

- Source-individual blood is tested for HBV, HCV, and HIV as soon as feasible after consent is obtained. Testing requires documented consent where required by state law; the exposed employee's need to know does not waive consent requirements.
- The exposed employee's baseline blood is collected with consent; if the employee declines testing, the sample is preserved for at least 90 days.
- Post-exposure evaluation, follow-up, and any prophylaxis are provided confidentially and at no cost to the employee.
- Hepatitis B vaccination is offered to all at-risk staff within 10 working days of initial assignment.

6. Severity & Escalation

A sharps/exposure incident is classified under POL-RM-013. A confirmed high-risk source (known HIV/HCV positive) or any exposure requiring PEP is escalated as SEV-2; routine low-risk exposures are SEV-3. Escalate immediately if the employee is symptomatic or the source is high-risk.

7. Documentation Requirements

- Route and depth of exposure; device type and brand, and whether a safety feature was engaged.
- Body fluid involved and estimated volume/duration of contact.
- Source identity and infectious status (if known); consent for source testing.
- Department/work area and a description of how the incident occurred — recorded on the OSHA Sharps Injury Log, kept confidentially for 5 years.

8. References

- OSHA Bloodborne Pathogens Standard, 29 CFR 1910.1030 (post-exposure evaluation §(f); sharps injury log §(h)(5)).
- CDC, Updated U.S. Public Health Service Guidelines for Management of Occupational Exposures to HIV and Recommendations for PEP.
- Related: POL-RM-013 (severity & reporting).